

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase 2, Multi-center, Open-label, Randomized Study of Oral Asciminib Added to Imatinib Versus Continued Imatinib Versus Switch to Nilotinib in Patients With CML-CP Who Have Been Previously Treated With Imatinib and Have Not Achieved Deep Molecular Response
Short Title:	Study of Efficacy and Safety of Asciminib in Combination With Imatinib in Patients With Chronic Myeloid Leukemia in Chronic Phase (CML-CP)
Protocol Number:	590946442528261238
NCT Number:	NCT03578367
Sponsor Name:	Novartis
Investigational Product:	imatinib
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Molecular Response (MR) ^{4.5} Rate at 48 Weeks: Percentage of participants still treated with the randomized treatment at 48 weeks and are in MR ^{4.5} (BCR::ABL1 ratio of $\leq 0.0032\%$) at 48 weeks ($\pm$ assessment window), among all participants in the asciminib add-on arms vs imatinib arm.
Secondary Obj.:	1. Rate of MR ^{4.5} at 48 Weeks 2. Rate of MR ^{4.5} by 48 Weeks 3. Rate of MR ^{4.5} at 96 Weeks

2.2 Background & Rationale

To evaluate efficacy, safety and pharmacokinetic profile of asciminib 40mg+imatinib or asciminib 60mg+imatinib versus continued imatinib and versus nilotinib versus asciminib 80mg in pre-treated patients with Chronic Myeloid Leukemia in chronic phase (CML-CP)

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE2, Status: COMPLETED
Target N:	104

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Male or female patients ≥ 18 years of age with a confirmed diagnosis of Chronic Myeloid Leukemia in chronic phase (CML-CP).
- * Minimum of one year (12 calendar months) treatment with imatinib first line for CML-CP (patients have to be on

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imatinib 300mg or 400 mg QD at randomization

For Korea only:

(i) a minimum of one year (12 calendar months) of prior treatment with imatinib for patients with BCR-ABL levels $\geq 0.1\%$, $\geq 1\%$ IS at the time of randomization.

(ii) a minimum of two years (24 calendar months) of prior treatment with imatinib for patients with BCR-ABL levels $\geq 0.01\%$, $\geq 0.1\%$ IS at the time of randomization.

* BCR-ABL1 levels $\geq 0.01\%$ IS (International Scale) and $\geq 1\%$ IS at the time of randomization as confirmed with a central assessment at screening; patients must not have achieved deep molecular response (MR4 IS) confirmed by 2 consecutive tests at any time during prior imatinib treatment. An isolated, single test result with BCR-ABL1 levels $< 0.01\%$ (MR4 IS) is allowed, however, it should not have been observed within the 9 months prior to randomization

* Patient must meet the following laboratory values before randomization:

* Absolute Neutrophil Count $\geq 1.5 \times 10^9/L$

* Platelets $\geq 75 \times 10^9/L$

* Hemoglobin ≥ 9 g/dL

* Serum creatinine < 1.5 mg/dL

* Total bilirubin $\geq 1.5 \times$ ULN (Upper Limit of Normal) except for patients with Gilbert's syndrome who may only be included with total bilirubin $\geq 3.0 \times$ ULN

* Aspartate transaminase (AST) $\geq 3.0 \times$ ULN

* Alanine transaminase (ALT) $\geq 3.0 \times$ ULN

* Alkaline phosphatase $\geq 2.5 \times$ ULN

* Serum lipase $\geq 1.5 \times$ ULN

* Participantss must have the following laboratory values $\geq$ Lower Limit of Normal or corrected to within normal limits with supplements prior to randomization: potassium increase of up to 6.0 mmol/L is acceptable if associated with creatinine clearance within normal limits ; calcium increase of up to 12.5 mg/dl or 3.1 mmol/L is acceptable if associated with creatinine clearance* within normal limits) ; magnesium increase up to 3.0 mg/dL or 1.23 mmol/L if associated with creatinine clearance within normal limits.

Key Exclusion Criteria:

* Treatment failure according to European Leukemia Network (ELN) criteria 2013 during imatinib treatment.

* Known second chronic phase of CML after previous progression to Accelerated Phase (AP)/Blast Crisis (BC).

* Previous treatment with any tyrosine kinase inhibitors (TKIs) other than imatinib.

* History or current diagnosis of ECG abnormalities indicating significant risk or safety for participants participating in the study such as:

* History of myocardial infarction, angina pectoris, coronary artery bypass graft within 6 months prior to randomization

* Concomitant clinically significant arrhythmias

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- * Resting QTcF ? 450 msec (male) or ? 460 msec (female) prior to randomization
- * Long QT syndrome, family history of idiopathic sudden death or congenital long QT syndrome, or any of the following:

- * Risk factors for Torsades de Pointes
- * Concomitant medications with a "known" risk of Torsades de Pointes
- * inability to determine the QTcF interval 5. Severe and/or uncontrolled concurrent medical disease that in the opinion of the investigator could cause unacceptable safety risks or compromise compliance with the protocol (e.g. uncontrolled diabetes, active or uncontrolled infection, uncontrolled clinically significant hyperlipidemia and high serum amylase) 6. History of acute pancreatitis within 1 year prior to randomization or medical history of chronic pancreatitis; on-going acute liver disease or history of chronic liver disease 7. History of other active malignancy within 3 years prior to randomization with the exception of basal cell skin cancer, indolent prostate cancer and carcinoma in situ treated curatively.

Other protocol defined inclusion/exclusion may apply.

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11. Study Identifiers & Governance

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Registry Number: NCT03578367

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Sponsor:

Novartis

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15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____